Member Update: AMA Victoria and ASMOF Victoria Enterprise Bargaining Campaign 2025 – Unified Log of Claims with Explanations

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Dear Members

As we commence negotiations in August 2025 for a new enterprise agreement covering all public hospital-employed doctors in Victoria, AMA Victoria and ASMOF Victoria have developed a comprehensive log of claims which has been strongly endorsed by members. These reflect your priorities and experiences across the system and are designed to secure fair pay, safe work, and a sustainable future medical workforce.

This document outlines the key claims and the rationale for each, drawing on evidence and feedback from members across Victoria.

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1. Key Wage Increase

Claim: 30% pay and allowance increase over four years.

Rationale: Vic Govt gave a 28% wage increase to Nurses in 2024 and doctors need at least the same increase just to maintain the relative value between Nurses and Doctors.
A 30% pay increase will also address inflation and supports retention.

Retaining doctors with competitive pay (when compared to other states) is essential to address chronic staffing shortages and ensure patient continuity of care and junior doctor training. Without regular and sufficient wage increases, doctors' real incomes decline. Retaining experienced doctors reduces reliance on locums, ensures quality of care and maintains training standards and provides incentives to work in rural and remote areas.

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2. Remuneration and Allowances

Claim: Equal pay (one hourly rate) for full-time and fractional specialists (move all doctors to 17.6 + classification rates).

Rationale: Removes disincentive for full-time roles.

Claim: Eliminate the "Full-time Specialist receiving Private Practice Income" lower rate.

Rationale: Outdated and no longer relevant due to changed billing practices.

Claim: Clause 31 to allow Above-Agreement payments where required.
Rationale: Enables flexibility to attract and retain doctors in high demand specialties (especially when considered against private practice).

Claim: Introduce penalty interest (10%) on delayed backpay beyond 3 months.
Rationale: Discourages employer delays in backpay claims and supports timely wage justice.

Claim: Improve payslip formatting and transparency.

Rationale: Allows doctors to verify payments and supports auditability.

Claim: Define that additional hours worked do not require prior approval to be paid as overtime.
Rationale: Prevents hospitals from avoiding overtime pay due to administrative pre-approval processes.

Claim: Remove clause 44a.2(b) linking Manager Allowance to additional hours or pay.
Rationale: Simplifies entitlement and removes unfair performance requirements.

Claim: Increase meal allowance to reflect actual costs (e.g. $25).
Rationale: Provides fair reimbursement for on-duty meals.

Claim: Maintain salary if moving from MO to Registrar or vice versa.
Rationale: Prevents inappropriate pay cuts based on administrative reclassification.

Claim: Pay higher duties for all hours HMOs act as Registrars.
Rationale: Fair recognition for acting-up responsibilities and reduces higher duties pay disputes.

Claim: Increase all shift penalties significantly.
Rationale: Reflects the inconvenience of shift work and encourages a more positive attitude to working unsociable shifts.

Claim: Triple time for shifts ending after midnight or starting before 6am.
Rationale: Addresses impact of overnight work on fatigue and wellbeing.

Claim: Double time for shifts ending 6pm–midnight on weekends.
Rationale: Recognises and goes some way to compensating for the burden of working weekend evenings.

Claim: Recognise all saved local agreements, including those not Fair Work Commission-certified.
Rationale: Maintains continuity of local arrangements and avoids disadvantage to doctors in services with pre-existing conditions.

3. Agreement Structure, Coverage and Purpose

Claim: One agreement to cover all doctors in public hospitals—DiTs, Specialists, Fellows, Assistants in Medicine, Ambulance Victoria and Adult Retrieval Victoria doctors.

Rationale: There should be only one agreement which covers all doctors employed in the public health sector. Having a single agreement will make the transition from DiT to Specialist seamless as well as providing more uniformity or commonality in the terms of the agreement applying to all doctors.

The current agreements do not cover all public health services and this needs to be rectified so that every public health service in Victoria is covered by the one agreement.

The current agreements allow for some doctors to be excluded from the agreement. The new agreement needs to cover all doctors employed in the public health sector. This includes AV and ARV.

A unified agreement ensures consistency of entitlements, consistency of entitlement interpretation and streamlines the transition between career stages and ensures that doctors receive equitable treatment across the system, locations and in all stages of their careers.

The current situation of 2 agreements leads to inconsistent conditions, confusion around entitlements, variable pay and leave conditions, unclear obligations and operational inefficiencies which all create inequities for doctors in the public sector.

Consistency provides doctors with clarity about their conditions, regardless of employer, location or stage of career; ensuring predictability and confidence.

Claim: Include all public health services and Ambulance Victoria in coverage.

Rationale: Ambulance Victoria applies the agreements to doctors it employs adding Ambulance Victoria to the list of employers covered by the Agreement will formalise what already happens in practice and will ensure consistent treatment of entitlements when a doctor moves between AV and Health Services or has fractional appointments with both.

Claim: No doctors to be exempt from agreement coverage.

Rationale: Prevents use of contracting models or job titles to avoid obligations.

Claim: The Agreement should use non-gender specific language (e.g. replace "his or her" with "they").

Rationale: Promotes inclusivity and aligns with best practice drafting conventions.

Claim: Add an Intent clause highlighting system-wide coverage, anti-bullying and anti-discrimination values, and rural workforce promotion.
Rationale: Embeds shared purpose, encourages cultural change and aligns the agreement with broader public sector values.

Claim: Include doctors in non-traditional training pathways (e.g. Rural generalists) to participate in rotation entitlements.
Rationale: Equity issue. Doctors are required to relocate to fulfill training requirements but do not have a "parent" hospital so are not currently covered by the Rotation clause in DiT Agreement and so have to bear the financial burden.

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4. Definitions and Classifications

Claim: Introduce or clarify definitions for Experience, Unaccredited Registrar, Registrar, Fellow, On-call, Clinical Academic, and Senior Medical Officer.
Rationale: Reduces disputes, clarifies entitlements and ensures correct classification and payment. This also reduces the burden on HR and Payroll teams to interpret vague clauses. Clear definitions avoid ambiguity and avoid disputes.

Claim: Define "Fellow" as a specialist-qualified doctor undertaking further training.
Rationale: Prevents Fellows being paid incorrectly as Registrars.

Claim: Define "Assistant in Medicine" (AiM), a final-year med student role, and create a new classification with appropriate pay.
Rationale: Supports safe practice and ensures fair recognition of AiM workers including pay rates.

Claim: Redefine "Rotation" to include college-directed rotations without Parent Hospitals.
Rationale: Ensures full access to entitlements for rural generalist and decentralised pathways.

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5. Employment Security and Contracts

Claim: Permanent employment to be the default. Fixed term only for backfill or externally funded projects.
Rationale: Supports job security and professional stability, addressing churn and workforce planning challenges.

Claim: Compliance with clause procedures before reporting to Colleges/AHPRA or issuing negative end-of-rotation reports.
Rationale: Protects junior doctors from unfair or premature escalation and supports procedural fairness. This change improves clarity and reduces the likelihood of disputes.

Claim: Certificate of service must include casual dates and submission of CoS to a new employer after 5 weeks is still valid.
Rationale: Reduces the number of doctors missing out on service-based entitlements.

Claim: Update redundancy clause to use LSL Continuity of service provisions when calculating payment.
Rationale: Recognition of doctors total service within the public health system.

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6. Hours of Work

Claim: Minimum engagement for Fractional doctors of 5 hours per session.
Rationale: Ensures stability and meaningful part-time arrangements.

Claim: Standard full-time hours for all doctors to be 38 hours per week, with specialists aligned to 35 hours where applicable.
Rationale: Promotes consistency and reflects actual working patterns and comparable professions.

Claim: Paid overtime for all hours worked beyond rostered shifts.
Rationale: Eliminates unpaid work and supports safe workloads.

Claim: No averaging of hours unless part of a formal averaged roster (e.g. 7-on/7-off).
Rationale: Prevents manipulation of rosters to avoid overtime payment and ensures transparency.

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7. Rostering

Claim: Maximum shift length of 12.5 hours.
Rationale: Aligns with fatigue science and promotes safe working conditions.

Claim: At least 4 rostered days off per fortnight. Two must be consecutive.
Rationale: Promotes and protects rest and recovery.

Claim: No rosters to include night shifts immediately before/after annual leave or relocation.
Rationale: Reduces exhaustion and relocation disruption. This change improves clarity and reduces disputes.

Claim: Annual balancing of night/weekend duties across teams.
Rationale: Ensures fairness, prevents overburdening individuals and promotes health and safety and rest and recovery.

Claim: Overtime paid for late roster changes or failure to provide 28 days' notice.
Rationale: Compensates for failure to provide rosters and promotes the timely provision of new rosters.

Claim: Maximum 4 consecutive night shifts.
Rationale: Limits fatigue and supports recovery.

Claim: 4 days off when coming off night shift before new rotation; 2 days off prior to relocation.
Rationale: Reduces fatigue and promotes safety during rotation and location changes. This eliminates disputes when employers roster doctors on the night before a rotation change or in days preceding a rotation change which also includes a service and/or location change.

Claim: Employer-provided accommodation during rotations must allow pets.
Rationale: Increases willingness of doctors to rotate to rural locations. This change improves doctor mental health and eradicates disputes.

Claim: Clinical Support Time for Specialists treated like DiT training time—paid as overtime if not provided.
Rationale: Recognises supervision, training time, and non-clinical time as work for senior doctors too.

8. On-call and Recall

Claim: On-call allowance increased to reflect the burden and frequency of call.

By the following changes;

> Standardized on-call arrangements for all doctors.
> On-call limited to 10% of ordinary hours.
> Create 2 On-call types: remote (within 4 hrs) and immediate (within 1 hr).
> Accommodation must be provided where a doctor is required to be immediate on-call (where accommodation is not available then an allowance will be paid to cover cost of alternate accommodation based on actual costs.)
> Max on-call period: 12 hrs.
> On-call compensation:
o 5 hrs pay Mon–Fri
o 8 hrs Sat–Sun
o 16 hrs on Public Holidays
> On-call must not cover staff shortages.
> 12 hrs rest after each on-call period.

Rationale: Ensures proper recognition of the burden of cover. This change improves clarity and reduces disputes.

Claim: Clearer recall provisions, including pay from time of call.

> Minimum 4-hr recall payment.
> Standard remote recall pay:
o First call (hour) instance: 1.5x
o Subsequent calls: 2x
o Public Holidays: 2.5x

Rationale: Ensures transparency and proper payment for work performed.

Claim: On-call not to be rostered the night before exam or conference leave.
Rationale: Supports educational and professional development.

9. Safe Work

Claim: Minimum doctor-to-patient ratios for safe care.
Rationale: Promotes patient and doctor safety, safe supervision and workload management.

Claim: Mandated paid uninterrupted rest break of at least 30 minutes for every 5 hours worked.
Rationale: Supports fatigue management and physical wellbeing.

Claim: Clinical Support Time (CST) to be better defined and protected, paid as overtime if not provided.
Rationale: Recognises this time as essential to perform all manner of tasks that are not directly related to patient care.

Claim: Space must be provided for overnight rest when rostered.
Rationale: Ensures fatigue mitigation and patient and workplace safety.

Claim: Recognise any on-call period as on-duty, not a rostered day off.
Rationale: Reflects the impact and constraints of being on-call.

Claim: Provision of overnight rest facilities when rostered.
Rationale: Mitigates fatigue and ensures safe practice for doctors on extended shifts.

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10. Leave Entitlements

New Leave Provisions

Claim: Donor leave, Reproductive, Gender Affirmation, Ceremonial and Occupational Violence leave entitlements to be included.
Rationale: Reflects modern, inclusive workplace values and aligns with members expectations.

Claim: Introduce new leave category for occupational violence.
Rationale: Supports recovery and wellbeing for doctors affected by workplace violence.

Claim: Public holiday rates to be paid on any work on a public holiday (including recall and overtime).
Rationale: Ensures proper compensation for holiday work and closes current loopholes. This will reduce disputes.

Personal Leave

Claim: Personal/carers leave continuity across casual and permanent roles.
Rationale: Avoids loss of entitlements due to insecure work arrangements. This change aligns with members expectations.

Claim: No requirement for statutory declaration or medical certificate for single-day personal leave.
Rationale: Trust-based model; reduces unnecessary administrative burdens on both the doctor and Payroll.

Claim: Compassionate and personal leave extended to cover pets and support animals.
Rationale: Reflects genuine relationships and grief. This change aligns with members expectations.

Claim: Clarify all accrued personal leave reinstated after casual service if returning to permanent role.
Rationale: Prevents loss of entitlements due to changes in service.

Parental Leave
Claim: 26 weeks paid per parent; no primary/secondary carer distinction; applies across contracts.
Rationale: Aligns with best practice for gender equity. This change aligns with members expectations.

Claim: Remove the term "contemporaneously" and redefine "child" to include child of legal surrogate.
Rationale: Removes outdated concepts and supports inclusivity. This change aligns with members expectations.

Claim: Increase paid partner leave.
Rationale: Promotes equity and supports families. This change aligns with members expectations.

Claim: Casuals entitled to paid parental leave at casual rate.
Rationale: Promotes equity. This change aligns with members expectations.

Claim: Provide affordable employer-supported childcare options.
Rationale: Supports parents to stay in the workforce. This change aligns with members expectations.

Claim: Remove 6-month qualifying period for paid parental leave. Entitlement continues if a doctor transitions to a new contract.
Rationale: Ensures access to parental leave for insecure workers, doctors who commence first contracts in Victoria and are unexpectedly pregnant and across contracts. This change aligns with members expectations.

Long Service Leave

Claim: Service recognition based on clinical years, not calendar years.

Rationale: A doctor is entitled to LSL after 7 years and it is necessary for the new Agreement to specifically provide that the entitlement is based upon 7 clinical years and not calendar years to avoid disputes over service.

Claim: Ensure LSL accrual and portability across all services by introducing portable LSL funding scheme.

Rationale: Prevents disadvantage for rotating or relocating doctors and doctors working across multiple services.

Claim: Change to 1/30th accrual; recognise all government service including interstate.

Rationale: Modernises system and supports portability.

Claim: Allow fractional LSL access across multiple services concurrently.

Rationale: Reflects real-world practice.

Claim: Sick leave can substitute during LSL if medically unfit.

Rationale: Protects entitlements so they are used for the reason they were created.

Claim: Portability of LSL across states and health services.

Rationale: Many doctors move between jurisdictions and health services throughout their careers. Ensuring portability of long service leave (LSL) entitlements recognises this mobility and avoids penalising doctors who contribute long-term to the public health system. It encourages retention within the broader public sector and reduces burnout by ensuring access to rest after extended service.

Claim: Change averaging of service from 2 years to total employment.

Rationale: Averaging service over the entire period of employment provides a more accurate and equitable measure of a doctor's entitlement, particularly where work hours have varied. The current two-year averaging method can disadvantage doctors who have worked full-time for many years but reduce their hours shortly before taking leave.

Claim: 13-week gap allowed between DiT and Specialist roles.

Rationale: Career transitions between DiT (Doctor in Training) and Specialist roles often involve short breaks for exams, relocation, or onboarding. A 13-week allowable gap prevents unfair loss of continuity for LSL entitlements and supports smoother transitions between key stages of medical careers.

Claim: Remove SMS 55.5(b) and DIT 68.5(b) to make LSL transfer automatic.
Rationale: These clauses currently require administrative action to initiate LSL transfer, which can be overlooked or inconsistently applied. Removing them ensures automatic, seamless transfer of entitlements and avoids unnecessary delays or disputes. It protects doctors' rights and simplifies compliance for employers. This change ensures that doctors are not treated unfairly.

Annual Leave

Claim: Increase annual leave to 5 weeks for all doctors.
Rationale: Aligns with entitlements for nurses and other health professionals; recognises intense workloads and improves work-life balance.

Claim: Ensure the agreement is clear that the doctor has the right to take annual leave at a time of the doctor's choosing. Hospitals cannot require a doctor to book all their leave prior to starting their contract.
Rationale: Ensures genuine flexibility and prevents pre-emptive rostering out of leave. This change aligns with member expectations, reduces disputes and improves morale.

Claim: If leave is not declined within timelines, it is deemed to have been approved.
Rationale: Promotes timely leave approval processes and protects access to leave.

Claim: Leave loading extended to all doctors.
Rationale: Corrects inequity across classifications and applies standard public sector practice.

Claim: Reinsert the annual leave payment calculation clause.
Rationale: Fixing error in current agreement that saw the clause removed.

Claim: Extra annual leave for rural service and option to purchase additional leave.
Rationale: Incentivises rural work and supports wellbeing. This change improves aligns with rural and remote workforce expectations.

Claim: Increase annual leave entitlements for weekend and shift workers.
Rationale: Recognises that weekend and irregular shift workers have higher fatigue loads and reduced access to time off aligned with their social and family lives.
Claim: Allow purchase of additional annual leave through salary sacrifice.
Rationale: Increases flexibility and supports work-life balance for those who prefer more time off over additional income.

11. CME and Career Development

Claim: CME unspent funds to be cashed out after 24 months.

Rationale: Stops health services creating barriers to spending CME entitlement to support their own budgets.

Claim: Reimbursement test based on reasonableness, not internal policy limits.

Rationale: Ensures doctors aren't financially disadvantaged by employers creating barriers designed to reduce monies paid when often the arbitrary limits bear no relationship to actual costs incurred.

Claim: Ensure equitable access to CME leave for all doctors, regardless of classification.

Rationale: Prevents exclusion of Carer Medical Officers and unaccredited registrars.

Claim: Count non-continuous time in registrar roles toward experience.

Rationale: Fairer recognition of real-world career progression. This will reduce disputes and improve morale.

Claim: Treat unaccredited registrar experience equal to accredited for service/time purposes.

Rationale: Reduces inequity between accredited and unaccredited doctors.
This will reduce disputes and improve morale.

Claim: CME childcare allowance indexed, allowance for larger accommodation when travelling with child (not capped by policy).

Rationale: Supports doctors with parenting responsibilities and addresses real costs.

Claim: Sabbatical Leave may only be refused or modified on grounds unrelated to work, with access subject to the agreement's dispute resolution procedure.

Rationale: Supports long-term retention, development, and research by enabling extended professional growth opportunities. Limits arbitrary refusals and ensures doctors have access to review mechanisms if sabbatical leave is denied.

12. IMG and OTP Support

Claim: Improved orientation, leave support, and assessment pathways for IMGs and OTPs.

Rationale: Assists doctors to become familiar with the Victorian system and successfully navigate it on their way to general registration.

Claim: Reclassify IMG doctors following general registration.
Rationale: Reflects full medical registration status. This change promotes fairness and equity.

Claim: Establish dispute resolution process for classification decisions.
Rationale: Promotes transparency and fairness. This change promotes fairness and equity.

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13. Career Medical Officer Pathway

Claim: Dedicated training and career pathway for CMOs.
Rationale: Recognises the long-term contribution of CMOs and supports career progression. This change reflects the increasing numbers of doctors who remain in the public system but do not specialise.

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14. Redundancy, Dispute Resolution, and Protections

Claim: Expand redundancy and redeployment entitlements for all classifications.
Rationale: Provides security during restructuring.

Claim: Strengthen dispute resolution clauses.
Rationale: Enables timely and fair conflict resolution.

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15. Union Rights and Representation

Claim: Paid time for union delegates and OHS Reps to attend training and represent members.
Rationale: Supports effective member advocacy and workplace democracy and safety.

Claim: Expand right of entry and consultation provisions.
Rationale: Ensures doctors can be represented and consulted effectively.

Claim: Hospitals must notify AMA/ASMOF of key contacts biannually or on change.
Rationale: Ensures union representatives have up-to-date and accessible contacts.

Claim: Include separate OHS clause that requires health services to have doctors represented in OHS committees, to be monitored through BPEC.
Rationale: Doctors are currently not represented on health service OHS committees.

16. Policies and Procedures

Claim: No policy to override or undermine the Agreement.

Rationale: Maintains enforceability and clarity. This clearly states what is ndustrial law – the Agreement overrides all policies (and a policy that overrides or undermines the Agreement is void).

Claim: Certificate of service on separation to be mandatory.

Rationale: Supports continuity of entitlements across employers and roles. This change reduces administrative burden on doctors.